NHS

# NHSE Delegated Commissioner Assignment – SQL Validation Tool

## Technical Guidance

## April 2025

Arden&GEM
Health and social care systems support

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# Supporting Guidance

**Material to support and validate the allocation of delegated activity relating to Specialised Services**

## 1.0 Introduction

1.1 In December 2024, the NHS England Board approved plans to delegate all suitable and ready specialised services to all ICBs from April 2025.

1.2 To support the process of service delegation, healthcare providers will be required to adopt the 2025/26 Commissioner Assignment Method (CAM) to assign the appropriate commissioner for healthcare activity reporting. Full details are available in the Commissioner Assignment Method (CAM) [https://www.england.nhs.uk/publication/commissioner-assignment-method-2025-26/] and the 'Who Pays?' guidance [NHS England » Who Pays?].

## 2.0 Scope

2.1 This tool **is optional and is not intended to replace existing processes**, but rather to support the validation of the responsible Commissioner allocation in accordance with the delegation of specific service lines submitted within the ACM and PLCM contract monitoring datasets.

2.2 The purpose of this document is to serve as reference material to support the delegation steps outlined in the national guidance. It also provides predefined logic to assist in implementing changes to the commissioner allocation methodology. The logic focuses specifically on changes resulting from delegation, rather than addressing the entire CAM process.

2.3 The attached SQL logic can be applied at the point where current processing concludes, using the delegation status of NHSE Service Lines and ICBs to identify and validate the appropriate commissioner for allocation.

2.4 Two versions of the validation process are attached for each dataset:

| 1. ACM Submission | 2. PLCM Submission |
|---|---|
| sproc_CAM_v5_3 (ACM).sql | sproc_CAM_v5_3 (PLCM).sql |

The logic contained within both the ACM and PLCM versions is identical, as the fields required for validation are consistent across both data submissions.

The fields required to support this methodology are listed below:

| Field Name | Narrative |
|---|---|
| [RECORDIDENTIFIER] | REPLACE [RecordIdentifier] with your row ID for the activity to enable linkage back to ACM/PLD. Note: This is not a Local Patient ID. |
| [COMMISSIONED SERVICE CATEGORY CODE] | Field direct from specification |
| [ARMED FORCES FLAG] | The logic is set to use the service category, but Providers are encouraged to ensure that this flag aligns with their current processing |
| [PUBLIC HEALTH FLAG] | The logic is set to use the service category, but Providers are encouraged to ensure that this flag aligns with their current processing |
| [HEALTH IN JUSTICE FLAG] | The logic is set to use the service category, but Providers are encouraged to ensure that this flag aligns with their current processing |
| [FINANCIAL MONTH] | Field direct from specification |
| [FINANCIAL YEAR] | Field direct from specification |
| [SERVICE CODE] | Field direct from specification |
| [ORGANISATION IDENTIFIER (CODE OF PROVIDER)] | Field direct from specification |
| [ORGANISATION IDENTIFIER (GP PRACTICE RESPONSIBILITY)] | Field direct from specification |
| [ORGANISATION IDENTIFIER (RESIDENCE RESPONSIBILITY)] | Field direct from specification |
| [GENERAL MEDICAL PRACTICE CODE (PATIENT REGISTRATION)] | Field direct from specification |
| [ACTIVITY TREATMENT FUNCTION CODE] | Field direct from specification |
| [POINT OF DELIVERY CODE] | Field direct from specification |
| [TOTAL COST] | Optional to assess impact |

**IMPORTANT NOTE**: Please use current process to identify these supporting fields.

# Supporting Information – Reference Tables

The prerequisite reference tables below must be set up prior to the SQL validation logic being run (referenced in section 2.4).

(i) Reference table indicating which specialised services are suitable for delegation in 2025/26 [ Please refer to **NHS England service codes by year** table via this web link NHS England » Directly commissioned services reporting requirements ]

This reference table also includes historical 2024/25 Service Codes to support multi-year validation.

| Excel File | SQL Insert |
|---|---|
| ServiceCodes.xlsx | **ServiceCodes.sql** |
| Table referenced as [CAM_Ref].[ServiceCodes] within the logic. | |

(ii) Reference table to illustrate the mapping of (sub) ICBs to Specialised Services regional commissioning hubs. This information will be used to determine the regional commissioner of services identified as suitable for delegation but not yet ready for physical delegation.
This table also provides sub-ICB delegation status as at 24/25.

Note: All sub-ICB's will be eligible for delegation from 25/26 onwards.

| Excel File | SQL Insert |
|---|---|
| SubICBtoDirectComm.xlsx | **CAM_Ref SubICBtoDirectComm** |
| Table referenced as [CAM_Ref].[SubICBtoDirectComm] within the logic. | |

(iii) Outside of the scope of this process but included for completeness and to provide up-to-date reference information for Armed Forces and Health in Justice are reference tables to help identify population-based commissioning earlier in the existing CAM process.

| Excel File | SQL Insert |
|---|---|
| Ref_HandJ_CommissionerCode.xlsx | **CAM_Ref Ref_HandJ_Commiss** |

| | |
|---|---|
| ArmedForcesGP.xlsx | CAM_Ref ArmedForcesGP.sql |
| Table(s) referenced as [CAM_Ref].[ArmedForcesGP] and [CAM_Ref].[Ref_HandJ_CommissionerCode] within the logic | |

(iv) Reference table to illustrate the mapping of Providers to host (sub) ICB. This information will be used to determine the regional commissioner of services identified as suitable for delegation but not yet ready for physical delegation.

Note: This reference table contains Delegation Status relating to 24/25 only. All sub-ICBs's are responsible for delegated services in 25/26. This is taken into account in the CAM SQL logic.

| Excel File | SQL Insert |
|---|---|
| REF_Provider.xlsx | CAM_Ref REF_Provider.sql |
| Table referenced as [CAM_Ref].[ REF_Provider] within the logic. | |

(v) Reference table to map and identify the period of activity between and in year.
This table allows the CAM SQL to determine and effective date which is then referenced within other Reference tables.

| Excel File | SQL Insert |
|---|---|
| YearMonthBridge.xlsx | CAM_Ref YearMonthBridge.sq |
| Table referenced as [CAM_Ref].[YearMonthBridge] within the logic. | |

(vi) Reference table to map commissioner assignment reason.
There is a limited set of commissioner outputs based on the CAM logic. This table is used in conjunction with the embedded CASE statements within the CAM SQL.

| Excel File | SQL Insert |
|---|---|
| CommissionerAssignmentReason.xlsx | CAM_Ref CommissionerAssigr |
| Table referenced as [CAM_Ref].[CommissionerAssignmentReason] within the logic. | |

(vii) Reference table to flag services.

Service flags are used to identify specific service types with the contract monitoring activity which is then used within the embedded CASE statement of the CAM SQL.

| Excel File | SQL Insert |
|---|---|
| ServiceFlags.xlsx | CAM_Ref ServiceFlags.sql |
| Table referenced as [CAM_Ref].[ServiceFlags] within the logic. | |

# Supporting Information – Delegation Logic

Below is a mapping description of the CAM flowchart aligned to the T-SQL scripts provided in Section 2.3. The 2025/26 CAM flowchart can be obtained by referring to the following website [https://www.england.nhs.uk/wp-content/uploads/2025/03/cam-flowchart-2025-26.pdf]

**Boxes A, B WB-1, WB-2, WB-3, WB-10** need to be completed by the Provider prior to the CAM Validation Tool application as per the Commissioner Assignment Method Flow Chart.

Line numbers have been provided below as an indication and may vary depending on the way the code is imported. To turn on Line numbers within Microsoft SQL Server Management Studio, please click:

Tools > Options > Text Editor > Transact-SQL > General

And Tick Line Numbers.

**SQL Script (referenced in Section 2.3)**

Firstly, please refer to line 83/84 in the SQL code where the provider needs to enter the [RECORDIDENTIFIER] the row ID for the activity, this will allow linkage back to ACM and PLCM data sources on completion and output of the validation tool.

Delegated Mental Health activity references lines 186 – 188. NOTE: MH activity is outside the scope of CAM.

**Box's C, C-0a, C-0b** 'Is the activity record specialised or highly specialised care?' and 'Is the service a green service refers to lines 201 - 235 in the code. In the SQL code the Service Code column is checked for no NULLs and the Specialised Flag column for 1 and column 'ICBDelegation Status' contains AMBER then the CASE block statements are tested, and the appropriate comments are assigned to the SubICB_code and ICB_code.

**Box C2** 'Does NHS England have a contract with the specialised service provider?' Refer to lines 221 - 236 in the code.

**Box D** 'Is the activity for secondary dental care?' Refer to lines 237 - 260 in the code where it checks in the Commissioned Service Category Codes 51 & 55 for secondary dental care or Dental TFCs and if yes then applies the applicable ICB commentary in SubICB1.[SubICB_Code], subICB3.[SubICB_Code] and HostSubICB.[Host subICB] table/columns. (SQL Code Lines 254 - 258).

If codes 51, 55 not found then go to box E.

**Box E,** 'Is the patient part of eligible health and Justice population', Refer to lines 263 - 271 in the SQL code:- checks for 71 (NHS England – H&J) 75 (Delegated H&J – when the service has been formally delegated) if this codes are found then a CASE SQL conditional statement is applied

checking the Table/column HandJ.EnglandHealthandJusticeCommissioningHubCode for No Nulls, if none then apply 'E – Commissioner Assigned by GP Practice' (Line 269).

If codes 71,75 not found, then go to box F.

**Box F, '**Is the activity related to Public Health service described in the Section 7a agreement', the SQL lines of code 275 - 287 relate to this – the code checks for codes 81 & 85 for Public Health if YES then the applicable comments apply in SQL code lines 281 - 285.

If codes 81,85 not found, then go to Box H.

**Box H**, 'Is it other public health activity?' NOTE: No current reference table available to determine 'Other' public health activity and requires determination using local processes.

**Box I**, 'Is the activity for Infertility treatment?' NOTE: This can only be identified by CDS Diagnosis codes, these are not present in ACM or PLCM Datasets and requires determination using local processes.

**Box I-1, '**Does the activity meet NHS England eligibility for Armed Forces?' NOTE: No current reference table available to determine 'Other' public health activity and requires determination using local processes.

**Box J**, 'Is the patient registered with an English GP Practice that has 13Q?', SQL Code lines 302 - 305 or service category code 61 for Armed Forces.

If no go to Box C-5.

**Box C-5,** Is the service a specialised Green Service, SQL Code Lines 309 – 316. Checks for Service Code column is not showing NULL and also that Specialised Flag = 1 and ICBDelegationStatus is containing Green in the column Then **Box C-6 (**SQL 319 - 372) the block of code under Box C-6 checks for either the ProvisionalICBDelegationStatus for 'YES' then the contents of SUBICB_Code column applies, if 'NO' then the contents from the ODSCommHub column applies.

**IMPORTANT NOTE:** This logic applies solely to determining the commissioner for delegated services and does not extend to any other aspects of the CAM or data items outside of the ACM and PLCM data sets. It is intended specifically to support the validation of existing processes where applicable.

If you have any queries, please contact: **DPRUSupport@nhs.net**

# National Resources and Useful Links

**NHS England Integrated Care Board delegation status of specialised services**
Identify the delegation status of specialised services to specific integrated care boards (ICBs).
Delegation status of specialised services

**Contract Monitoring Information Standards**
Information standards for Contract Monitoring to enable national consistency of the flow of cost and activity information from providers to commissioners.
Contract Monitoring - NHS England Digital

**NHS England Service Codes**
Service codes and their delegation status in 2025/26 for all NHS England directly commissioned services and those for which it retains oversight.
NHS England » NHS England service codes by year 2025/26

**Commissioner Assignment Method (CAM) 2025/26**
Supporting information for the commissioner assignment method process for 2025/26.
NHS England » Commissioner assignment method 2025/26

**Who Pays?**
Framework for establishing which NHS commissioner will be responsible for commissioning and paying for an individual's NHS care.
Who Pays?

# Frequently Asked Questions

1) **The reference files are embedded in the word document and we are struggling to open them.**

   The logic contains table create and insert statements for all of the reference tables used in the process. We have also attached the reference tables as excel files to the email supporting this document.

2) **Will these checks be added to the Data Landing Platform (DLP)? If this logic is not applied before submission, will the submission fail?**

   These validation checks will not be implemented on to the DLP. The checks on the DLP are intended to test whether the submission file contains any data items or formats which would impact on the onward processing of the data.

   Not applying this logic prior to submission will not cause the submission to fail. This logic is aimed at improving the consistency of service category and commissioner code following delegation prior to submission but will not prevent submission.

3) **Will there be an opportunity to make a test submission before the first flex date?**

   The logic is a standalone process and a test submission is not required. The intended use is to process the existing Patient Level Contract Monitoring (PLCM) and Aggregate Contract Monitoring outputs locally to see where there are variances in service category or commissioner code. The validation tool provides a reason for every code allocation and these will help understand the part of the CAM logic which was used as the decision point and why the allocated code was selected.
   The aim is to focus on major discrepancies to reduce the variance.

4) **We know we have Data Quality (DQ) issues- will the tool be used to send back queries/ challenges?**

   The validation tool provides instant feedback on variances and can be used to identify areas for investigation. It is a simplistic approach and is unable to utilise all of the information providers have access to.

   The CAM logic being developed and standardised across all DSCROs, will review this in more detail and an additional DQ report is available for providers via the NCDR Portal to review variances in ACM, PLCM and Commissioning Data Set (CDS) data submitted to Secondary Uses Service (SUS).

5) **Will the DQ report be emailed out as I am struggling to access the NCDR?**

The DQ report will not be emailed out. If you have problems accessing content please contact the NCDR helpdesk:
ncdr@england.nhs.uk

**6) Will SUS be applying the new rules?**

Yes, development work is taking place by the national SUS team to implement updated rules.

**7) Should the process be applied to SUS data? We produce SUS outputs before ACM/ PLCM- How can we use this tool at an earlier stage of the process?**

For services which flow through both SUS and local flows, there is an expectation that activity records should match. Therefore, the CAM logic and associated rules as per the Who Pays Guidance should also be applied to SUS activity.

Provider processes are not standard and it is not possible to supply logic to cover every eventuality. Some providers take an extract of data which is then used for both SUS and local flow submissions. The tool has been built to use a subset of data fields from ACM or PLCM but can also be used on SUS data, as long as the key fields are included. The use of a record identifier enables the post processed data to be linked to the relevant input record for validation.

**8) Our system provider has stated that they will not be applying the rules in their processes- is this correct?**

The Commissioner Assignment Methodology (CAM) details the process to identify responsible commissioner and is required to be used by all healthcare providers and commissioners. The technical logic documented within the CAM has been partially replicated within the logic script produced by AGEM CSU. Use of the AGEM CAM validation tool script is optional for healthcare providers and their suppliers, but the healthcare provider (and system supplier) is expected to apply the technical logic documented within the published CAM.

9) **Does this logic account for top-up payments for delegated services?**

The logic does not currently treat top-up payments separately. Please see national guidance published on the NHSE website.

**10) Should Delegated services be submitted with a service code of 99999999?**

All Directly Commissioned activity, irrespective of whether the service is being delegated, should be submitted with the relevant NCBPS code. To clarify, all healthcare activity meeting the definition of a specialised service (as determined by the Prescribed Specialised Manual and / or associated Identification Rules ) must include a specialised service NCBPS code. This

detail ensures that service demand and supply can be identified and analysed in order to effectively manage the service nationally.

**11) Can you confirm that Delegated activity should be submitted with a sub-ICB code as the commissioner and a service category of 25?**

That is correct in certain situations, as set out in the CAM guidance. If the service is a green service in 25/26 the sub-ICB should be recorded as the commissioner and the service category would be 25.

The CAM logic and the supporting information provide guidance on what should be recorded against activity for each situation.

12) **Does this apply to the Emergency Care Data Set (ECDS)?**

No, this does not apply to ECDS as it falls outside the PSS Toolkit.

13) **Will this be built into existing products such as the PSS Toolkit? Is there potential for a Central CAM application rather than each provider building the logic?**

There is no immediate plan for this logic to be built into existing commissioning products. However, shared CAM logic is being developed by DSCROs to standardise the application of CAM following delegation to ensure that data is available to the correct commissioner. This development may provide assurance that a single national CAM application would be accurate and timely, reducing the need for provider specific implementation.

**14) Is there a similar tool for Mental Health Services Data Set (MHSDS)?**

This tool has not been developed for use with MHSDS data but it could be extended to include Directly Commissioned MH services. Arden and GEM CSU will liaise with our national mental health colleagues to explore this further.

**15) The output has activity which is allocated a Null Commissioner code but an ICB code has been populated. Why is this?**

This happens where the CAM logic requires a Sub-ICB location. We are unable to determine which Sub-ICB is correct within an ICB and so the commissioner code is set to Null.

**16) Where can I find out more information about delegation?**

Delegation status of specialised services
Information relating to delegation and Directly Commissioned services can be found on the link above.

17) **Can you share the CAM logic and flow diagram?**

Supporting information for the commissioner assignment method process for 2025/26 is available online
NHS England » Commissioner assignment method 2025/26

**18) Will the attached scripts make changes to our existing tables?**

No. The scripts build and populate tables used within the process itself. Users set the database that the scripts run in, and the supplied logic will create and populate reference tables and processing tables. Users will need to write code to extract the required fields into a temporary table from their existing ACM and PLCM output tables.

19) **If a GP practice is not known for a patient, the commissioner for delegated services should revert to hosted Provider region rather than utilise patient postcode?**

Delegation is a move towards population based commissioning and utilising patient postcode in the absence of a registered GP Practice code will more accurately reflect patient location than the host Provider region. This is especially true of Directly Commissioned services.

The general approach to commissioner identification is as follows:

1. GP Practice
2. Patient Postcode
3. Host Provider.

20) **Locally H&J is commissioned by ICBs, not NHSE. We can ignore the flag, but is it possible to amend the code?**

With a standard tool for use nationally, it is not possible to incorporate local rules. In this case, the variance can be noted locally- the tool is designed to suggest areas which may require further investigation rather than to be used as to overwrite all variances.

It would be possible for local IM&T staff to change the logic, but this may cause unintended consequences or make it more difficult to implement any future versions. However, the logic has been shared and documented for full transparency and can be used as a basis for provider logic if so desired.

**21) How can we be sure we are doing the same thing when we have been given the choice as to the approach we intend to take for DLP submission?**

The options for submission method have been reviewed by DSCROs to ensure they will facilitate getting the correct data to the correct recipient commissioner post delegation. As delegation itself requires the change of existing provider processes, the intention was to not create further work by changing the submission methods.

Furthermore, the CAM logic being applied to data submissions is consistent across all DSCROs which will ensure consistency of processed data outputs.

**22) Will there be further sessions for our IM&T colleagues?**

No further technical drop in sessions have been scheduled, if you require assistance please contact: **DPRUSupport@nhs.net**

# Change Log

April 2025 (v5.3)

**Financial Year and Month Field**

- This version of the CAM Validation Tool is dependent on the Financial Year and Financial Month fields being accurately populated as specified in the technical specification documentation.
- This supports handling of multi-year submissions and associated activity year rule logic.

**Identification of 'Blue' Service Lines**

- 'Blue' service lines refer to reporting lines where funding is managed by a host provider, such as in clinical network funding arrangements.
- These are subject to local contracting agreements and are out of scope for this validation tool.

**Health & Justice**

- Introduced identification logic for A&E patients within the Health & Justice context.

**Secondary Care Dental**

- Implemented assignment of *Commissioned Service Category Code 55 – DELEGATED SECONDARY DENTISTRY* where dental service lines are detected.

**Performance Improvements**

- Optimised T-SQL logic to improve performance, especially for large volumes of data and multi-year submissions.

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**Get in touch with us at:**

www.ardengemcsu.nhs.uk

@ardengem

contact.ardengem@nhs.net